

## BRANCH PROFITABILITY EDUCATION SERIES

# Why Your Census Number Is Not Arbitrary

What Nobody Tells Hospice Sales Reps About the Business They Sell For

A Spartan Coaching Field Education Document | Branch Profitability Series

*The number you were given without an explanation. This document gives you what your sales leader did not have time to explain, what your clinical team assumed you already understood, and what your executive director may not have known how to translate into language a rep in the field could actually use.*

## CONTENTS

Before the Numbers — What a Hospice Branch Is Actually Doing

Part One — The People Behind the Cost Structure

Part Two — What It Costs to Care for Each Patient, Every Day

Part Three — The Numbers That Should Have Been Explained on Day One

Part Four — Reading the Field Differently Now That You Know This

The Branch Profitability Simulator — A Tool Built to Make This Visible

What This All Means — The Census Is Not the Point. The Care Is.

Key Terms — Quick Reference

## BEFORE THE NUMBERS

## The Number You Were Given Without an Explanation

At some point early in your hospice sales career, someone gave you a number. Eight admissions a month. Twelve. An ADC of 60 by Q3. Maybe it came with a chart. Maybe it came with a warning. But what it almost certainly did not come with was an explanation of where it came from, what it costs the organization every single day it goes unmet, or — most importantly — what it actually means for the people your branch exists to serve.

That is not a small gap in your education. It is a fundamental one. Because without understanding the why behind the number, you are executing a task without understanding its purpose. And in hospice — where the work you do connects directly to whether a dying person receives expert, compassionate care in their final weeks of life — purpose is not optional context. It is the job.

***This document exists to give you what your sales leader did not have time to explain, what your clinical team assumed you already understood, and what your executive director may not have known how to translate into language a rep in the field could actually use.***

Read it once. Then read it again when the week is hard and the number feels abstract. Because the number is never abstract. Behind every point of ADC is a nurse making a 2 a.m. phone call, a social worker sitting in a family's kitchen, a chaplain holding someone's hand at the end of a long life. The census is how that work stays funded.

### What a Hospice Branch Is Actually Doing

Most people outside of healthcare think of hospice as a place you go to die. Most people inside healthcare know it is the opposite. Hospice is where you go to live — with comfort, with dignity, with a coordinated team of clinical professionals who take over the medical burden so the patient and family do not have to carry it alone.

When a patient enrolls in hospice, they are placing an enormous amount of trust in your branch. Not just trust in the idea of hospice, but trust in specific people. The RN case manager who will come to their home twice a week and know their name, their pain level, their fears, and their wishes. The hospice aide who will help them bathe and stay clean and feel human. The social worker who will navigate the family dynamics and help a daughter figure out how to stop being a medical coordinator and just be her father's daughter again. The chaplain who will show up without an agenda and sit with whatever the patient needs to say.

***"A patient does not just enroll in a benefit. They entrust a team of people with the most intimate chapter of their life — and their death."***

That team does not appear because of goodwill. It does not exist on hope or mission statements. It exists because your branch generates enough revenue from enough patients on census to employ, train, schedule, supervise, equip, and retain every one of those people. Every day.

**That is the real meaning of the census number. Not a line on a dashboard. Not a quota to satisfy. It is the financial floor below which that care team cannot be sustained — and below which the patients who need them do not receive them.**

## PART ONE

## The People Behind the Cost Structure

Every line item in a hospice budget is a person. Understanding the cost structure of your branch means understanding the people your census supports — and what happens to them, and to your patients, when that census falls short.

### The Clinical Team — The Heart of the Branch

These are the people who provide direct patient care. They are not optional staff. Medicare Conditions of Participation require that every hospice maintain these roles regardless of census. They must be hired, trained, credentialed, and retained — and their salaries are paid every two weeks whether the census is 20 or 80.

| Clinical Role & Purpose in Patient Care                                                                                                                                                                                                                                                                                   | Annual Cost    |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------|
| <b>Executive Director</b><br>Oversees clinical operations, compliance, and team performance. The accountable leader who ensures every standard of care is met and every regulatory requirement is satisfied.                                                                                                              | \$140,000      |
| <b>Supervisor RN Case Manager</b><br>Manages clinical staff and ensures care quality standards. Bridges the gap between administration and bedside — the person who catches problems before they reach patients.                                                                                                          | \$110,000      |
| <b>RN Case Manager (×2 minimum, +1 per 12 patients)</b><br>Primary clinical contact. Visits patients at home twice per week or more. Manages pain and symptoms, coordinates care, and is often the most trusted person in a dying patient's life outside of family.                                                       | \$100,000 each |
| <b>Hospice Aide (×2 minimum, +1 per 8 patients)</b><br>Personal care — bathing, hygiene, comfort, and human dignity. Often the clinical team member with the most frequent contact. Provides the hands-on care that keeps patients clean, comfortable, and connected to their sense of self.                              | \$50,000 each  |
| <b>Social Worker</b><br>Family support, care planning, grief counseling, and discharge navigation. Helps families stop managing logistics and start being present. Coordinates financial resources, communicates with facilities, and carries the emotional load that would otherwise fall on exhausted family members.   | \$75,000       |
| <b>Chaplain</b><br>Spiritual support, meaning-making, and presence at end of life. Shows up without an agenda and sits with whatever the patient needs to say — fear, regret, faith, doubt, or silence. Provides care for the dimension of dying that medicine cannot reach.                                              | \$70,000       |
| <b>After Hours RN</b><br>Responds to patient crises overnight — the 2 a.m. phone call that a family member is panicking because their loved one's breathing has changed. Ensures that no patient faces a crisis alone, and no family faces the fear of the unknown without a clinical voice on the other end of the line. | \$95,000       |

|                                                                                                                                                                                                                                                   |          |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------|
| <b>Weekend RN</b><br>Ensures continuity of care when the week ends for everyone else. A patient's need for pain management or symptom control does not observe business hours. This role ensures that care does not pause on Saturday and Sunday. | \$95,000 |
| <b>Medical Director (Contract)</b><br>Certifies eligibility, oversees pain and symptom management protocols, and provides physician-level oversight of every patient's care plan. Required by Medicare for every certified hospice.               | \$75,000 |

***As census grows, so does the clinical team — by Medicare requirement, not by choice. An RN Case Manager is typically added for every 12 patients on service. A Hospice Aide for every 8. The branch cannot delay those hires without risking compliance and, more importantly, without failing the patients already enrolled.***

*Below break-even, the branch absorbs the loss — and the clinical team feels it.*

What this means in practice: when census drops, these positions are the last to be eliminated — because eliminating them means abandoning enrolled patients. The branch absorbs the loss. It operates in the red. It draws on reserves. And the pressure falls on the sales team to restore the census that makes those positions financially viable again.

## The Operational Team — The Infrastructure That Makes Care Possible

Behind every patient visit is a scheduling system, a documented care plan, a supply order, a referral processed, and an admission coordinated. These roles exist so the clinical team can focus on patients instead of paperwork.

| Operational Role & What They Make Possible                                                                                                                                                                                                                                       | Annual Cost |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------|
| <b>Intake Coordinator</b><br>Receives referrals, processes admissions, and coordinates the first visit. Often the first person a new patient's family talks to after the referral is made. The quality of that first interaction sets the tone for the entire care relationship. | \$60,000    |
| <b>Secretary / Office Administrator</b><br>Scheduling, documentation, billing support, and office operations. The operational foundation that keeps the branch functioning — without which the clinical team spends their time on paperwork instead of patients.                 | \$55,000    |

## The Sales Rep — The Person Who Makes All of It Possible

Every other role on this list depends on what you do in the field. The RN Case Manager only has a patient to care for because a referral was made. The social worker only has a family to support because someone had the right conversation with the right physician at the right time. The chaplain only has a purpose in the branch budget

because census is high enough to support their position.

| Sales Role & What They Generate                                                                                                                                                                                                                                                                                                     | Annual Cost |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------|
| <b>Sales Rep / Market Development Liaison</b><br>Admissions, referral relationships, and census growth. The originating link in the chain of care. Every patient on census is there because a rep had a conversation, built a relationship, and followed through on a referral. Every position on this list is funded by that work. | \$115,000   |

***Your salary is a fixed cost paid whether you produce or not. But more than that — every role on this list is funded by the census you build. You are not chasing a quota. You are funding a care team.***

## PART TWO

## What It Costs to Care for Each Patient, Every Day

Beyond salaries, every patient enrolled in hospice generates a daily cost. These are the direct costs of delivering care to one person, one day at a time. They are variable — they scale with census — and they must be covered before a dollar of profit is possible.

| Daily Patient Cost               | What It Covers                                                                                                                                                                                                       | Per Day          |
|----------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------|
| <b>Pharmacy / Medications</b>    | Pain management, comfort medications, and symptom control drugs. Standard acuity patients average \$22–\$25/day. Oncology, advanced heart failure, or complex pain management patients can reach \$44–\$80/day.      | \$22–\$44        |
| <b>Durable Medical Equipment</b> | Hospital beds, wheelchairs, oxygen equipment, walkers, and other equipment that allows a patient to receive care at home rather than in a facility.                                                                  | \$10             |
| <b>Medical Supplies</b>          | Wound care supplies, nursing equipment, personal protective equipment, and other consumables used in home visits. High-acuity patients with complex wounds or multiple symptom needs push this significantly higher. | \$10–\$23        |
| <b>Clinical Travel</b>           | Mileage and time cost for home visits by RNs, aides, social workers, and chaplains. Varies by territory geography and patient density — rural branches carry significantly higher travel costs.                      | \$6              |
| <b>Other Patient Costs</b>       | Volunteer coordination, bereavement services, contracted therapy, interpreter services, and other per-patient expenses that do not fall cleanly into the categories above.                                           | \$5              |
| <b>Total Daily Variable Cost</b> | Per patient, per day — before salaries or overhead                                                                                                                                                                   | <b>\$53–\$88</b> |

High-acuity patients — those with advanced heart failure, ALS, end-stage COPD, or complex pain management needs — can push daily medication costs alone past \$80. These patients often have longer lengths of stay, which benefits the branch financially, but their daily care cost is significantly higher and must be factored into any honest financial model.

### Monthly Overhead — The Cost of Having a Place to Do This Work

In addition to salaries and direct patient costs, every branch carries a layer of fixed operational overhead that exists before anyone walks through the door. This includes office rent, utilities, liability insurance, professional licensing fees, billing and EMR software, compliance training programs, office equipment, and telephone systems — costs that have nothing to do with any specific patient but everything to do with being able to operate at all.

| Overhead Category                   | Monthly Cost (Approx.)   | Annual Total                      |
|-------------------------------------|--------------------------|-----------------------------------|
| Office Rent, Utilities, Insurance   | \$18,000–\$22,000        | \$216,000–\$264,000               |
| EMR / Billing / Compliance Software | \$6,000–\$8,000          | \$72,000–\$96,000                 |
| Training, Licensing, Credentialing  | \$4,000–\$6,000          | \$48,000–\$72,000                 |
| Administrative & Miscellaneous      | \$8,000–\$12,000         | \$96,000–\$144,000                |
| <b>Total Monthly Overhead</b>       | <b>\$35,000–\$45,000</b> | <b>\$420,000–\$540,000 / year</b> |

***Before a single salary is paid. Before a single patient medication is purchased. Before a nurse makes a single visit. The branch is spending between \$420,000 and \$540,000 per year just to exist as an operating entity. Census is what covers that cost — and everything above it.***

## Where the Revenue Comes From

Medicare pays hospice under a per diem model called Routine Home Care (RHC). For each day a patient is enrolled and receiving care, the branch receives a fixed daily payment from Medicare. This rate does not change based on how many visits were made that day or how much medication was administered. It is a flat daily rate — which is why census is the primary driver of revenue. More patients on service means more days of reimbursement.

| Medicare Payment Period     | Daily RHC Rate (Approx.)        | Why It Changes                                                                                                                                                                                                                      |
|-----------------------------|---------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Days 1–60 of a spell</b> | \$208–\$224 per patient per day | Higher rate reflects greater clinical intensity in the initial enrollment period, when symptom management, family education, and care plan establishment demand the most from the clinical team.                                    |
| <b>Days 61 and beyond</b>   | \$177–\$209 per patient per day | Rate adjusts downward after the initial high-intensity phase. Longer-stay patients are typically more stable, requiring less intensive clinical intervention — but they remain on service and continue generating per diem revenue. |

After subtracting the daily patient costs (\$53–\$88), the remaining amount is the contribution margin — the portion of each patient day that goes toward covering salaries, overhead, and ultimately profit. That contribution margin is approximately \$120 to \$155 per patient, per day.

***Every patient on census generates roughly \$120 to \$155 per day toward the fixed costs that keep your clinical team employed and your branch operational. Every vacant day — a patient who was never admitted, who left service early, who was never referred — is \$120 to \$155 that did not arrive.***

## PART THREE

## The Numbers That Should Have Been Explained on Day One

### Break-Even ADC — The Floor Below Which Everything Breaks Down

Break-even ADC is the Average Daily Census at which total revenue exactly equals total costs — salaries, daily patient costs, and overhead combined. It is not a goal. It is not a benchmark. It is the minimum viable condition for the branch to function without losing money every single day.

***A typical hospice branch requires between 35 and 55 patients on census at all times just to reach break-even. Below that threshold, the branch absorbs a daily operating loss. Those losses drain cash reserves, constrain clinical hiring, and create financial pressure that eventually affects the quality of care — even if no one says so out loud.***

What happens when a branch operates below break-even for an extended period? Leadership delays replacing a departing RN. After-hours coverage gets stretched thin. Supply orders get scrutinized. Training budgets get cut. The clinical team feels the pressure even if they cannot name its source. And the patients — the patients who enrolled because they trusted this organization — receive care from a team that is operating under financial strain.

**That is the human cost of a census that stays below break-even. It does not show up on a balance sheet. But it shows up in the field.**

$$\text{Break-Even ADC} = (\text{Annual Payroll} + \text{Annual Overhead}) \div (\text{Contribution Margin per Patient per Day} \times 365)$$

### Target Margin ADC — The Number That Actually Funds the Mission

Break-even means the branch survived the month. Target Margin ADC means the branch is sustainable, investable, and able to grow. Healthy hospice organizations operate at an operating margin of 12 to 18 percent. That margin is not profit extracted from patients. It is the financial cushion that allows the organization to:

- Hire additional clinical staff before census demands require it, so new patients receive full attention from day one.
- Invest in training and continuing education that keeps the clinical team sharp and current on symptom management.
- Build administrative infrastructure that reduces errors and delays in care coordination.

- Expand to underserved markets where eligible patients are not yet receiving care.
- Weather market disruptions, regulatory changes, and reimbursement adjustments without cutting clinical staff.

The difference between break-even ADC and target margin ADC is typically 15 to 30 additional patients on census. Those patients are not a rounding error. They are the difference between an organization that survives and one that thrives — and between a clinical team that is stretched and one that is supported.

***Every admission above break-even is not just revenue. It is investment capital for a better clinical operation, a stronger care team, and more patients reached in the future.***

## The Admissions Math — What You Actually Need to Produce

Census does not hold itself. Patients die. Some discharge. Some revoke. A branch must continuously admit new patients to replace those leaving service. The number of monthly admissions required is driven by two variables: your target ADC and your average length of stay.

$$\text{Monthly Admissions Needed} = (\text{Target ADC} \times 365) \div \text{Avg. Length of Stay} \div 12$$

| Target ADC   | Avg. Length of Stay | Monthly Admissions Needed | Weekly Needed |
|--------------|---------------------|---------------------------|---------------|
| 30 patients  | 60 days             | ~15 admissions / month    | ~4 per week   |
| 50 patients  | 90 days             | ~20 admissions / month    | ~5 per week   |
| 60 patients  | 90 days             | ~24 admissions / month    | ~6 per week   |
| 80 patients  | 90 days             | ~32 admissions / month    | ~8 per week   |
| 100 patients | 90 days             | ~41 admissions / month    | ~10 per week  |

***If your branch targets 60 ADC with a 90-day average length of stay, it needs approximately 24 new admissions per month — 6 per week — just to hold census flat. Not grow it. Hold it. If two marketers split that production, each rep needs to close 12 admissions per month. That is not a soft goal. That is the math behind maintaining the care team that is already employed.***

## PART FOUR

## Reading the Field Differently Now That You Know This

The conversations you have in the field look different when you understand what is at stake behind the census number. Here is how to reread situations you have already experienced.

**When your manager says 'we need three more admissions this month,'**

They may not be talking about a quota. They may be talking about the break-even line. Three admissions at a 90-day average length of stay adds 7 to 8 patients to the running ADC over the following quarter. At \$135 per patient per day in contribution margin, that is roughly \$340,000 in annual revenue that either arrives or does not. The urgency is real. Now you know why.

**When a referral stalls and you do not follow up,**

The cost is not a missed commission. It is a patient who did not get care, and a family that navigated end of life without a team behind them. It is also a gap in census that the branch absorbs financially — paying every salary on that list — while the revenue that would have covered it never arrived. Referrals do not stall themselves. They stall in the gap between your last contact and your next one.

**When a patient is admitted with a very short prognosis,**

A two-week patient barely covers their own admission cost after all direct expenses are calculated. The patients who sustain a branch financially — and who receive the most meaningful hospice care — are those who were referred early, when hospice had months to provide comfort, education, and family support. Early referral is not just a financial win. It is the difference between a patient who had time to know their care team and one who met them for the first time days before dying.

**When the clinical team seems stressed and understaffed,**

There is likely a direct connection to census. Below break-even, the branch may be holding open positions, stretching existing staff thin, or delaying hires that the caseload genuinely requires. The RN who has one too many patients on her list is a symptom of a census that is not sustaining the staffing model it needs. Your production is not separate from clinical quality. It is upstream of it.

**When you understand that every conversation you have connects to the census,**

The work becomes something different. Not a quota to survive. Not a metric to manage. A direct line between your effort in the field and the quality of care that patients receive in their homes. You are not moving referrals. You are funding the team that shows up at 2 a.m. You are sustaining the chaplain who sits with someone who has no family left. You are making it possible for a social worker to tell a family they do not have to do this alone.

## THE BRANCH PROFITABILITY SIMULATOR

## A Tool Built to Make This Visible

The Spartan Coaching Branch Profitability Simulator was built because this education almost never happens in the field. Sales reps are given goals without context. Leaders are given dashboards without explanations. And the connection between daily census, clinical staffing, patient costs, and care quality remains invisible to the people whose work most directly determines it.

| Tool Feature                         | What It Shows You                                                                                                                                                                                                                  |
|--------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Break-Even ADC</b>                | Enter your revenue rates, clinical costs, and staffing assumptions to calculate the minimum census that covers all expenses. See the exact number your branch must sustain before it stops losing money every day.                 |
| <b>Target Margin ADC</b>             | Set your operating margin goal (12–18% is healthy) and see exactly what census is required to achieve it. This is your real sales target — not just break-even, but the census that builds a sustainable, investable branch.       |
| <b>Full Staffing Cost Model</b>      | See how clinical and operational payroll scales as census grows, including which positions are triggered at which caseload thresholds. Understand the Medicare-required staffing increases that happen at every 8 and 12 patients. |
| <b>Daily Patient Costs by Acuity</b> | Model low, base, and high-acuity patient populations to understand how diagnosis mix affects financial performance. See how an oncology-heavy census changes your variable cost structure and contribution margin.                 |
| <b>Monthly Admissions Required</b>   | Convert your target ADC into a specific monthly admission number based on your actual average length of stay. Know the exact production number you and your team must hit — not an estimate, the math.                             |
| <b>Marketer Productivity Targets</b> | See how many admissions per marketer are needed to sustain the census goal. Understand what your individual production target means in the context of the team's overall need. The math visible, not assumed.                      |
| <b>Cash Runway Projection</b>        | Model how long starting capital can sustain a branch operating below break-even while census builds. Know the month your branch turns cash-flow positive and whether your ramp plan is realistic before you commit to it.          |
| <b>Profitability Curve</b>           | Visualize how profit and operating margin change across the full census range from 10 to 200 patients. See exactly where break-even occurs, where your target margin kicks in, and how far your current census sits from both.     |

***You have never had access to this information in a form you could use in the field. The simulator puts the complete financial picture of your branch in front of you — not as a report to review after the fact, but as a working model you can adjust, test, and use to understand the real stakes of your daily production.***

*Access the Branch Profitability Simulator at [spartancoaching.com](https://spartancoaching.com)*

## WHAT THIS ALL MEANS

## The Census Is Not the Point. The Care Is.

Everything in this document — the staffing model, the daily costs, the break-even math, the revenue rates, the target margin — exists to support a single reality: somewhere in your territory, right now, there is a person who qualifies for hospice care and is not receiving it. Their family is managing their medications without guidance. Their pain is being managed inadequately or not at all. They are facing the end of life without a team of professionals who know their name and their wishes.

That gap is not clinical. Physicians understand hospice. Nurses understand hospice. The gap is conversational. A referral that was not made because the rep did not have the right language. A physician who said 'not yet' because no one responded with clinical confidence. A patient who was never asked because the rep's calendar was full of low-value visits to accounts that were never going to convert.

***"The census is not the point. The census is what makes the point possible. The point is that a nurse shows up. A social worker calls. A chaplain sits. A family gets through the hardest days of their lives with people beside them who know exactly what they are doing and why they are there."***

Your work in the field is the beginning of that chain. Every conversation you prepare for. Every referral you follow through on. Every physician you educate, every discharge planner you support, every family you help get to the right decision at the right time — that work funds the clinical team that carries it forward.

**When you understand that, the number is not arbitrary. It never was. You just needed someone to explain it.**

### Key Terms — Quick Reference

| Term                              | Definition                                                                                                                                                                                                                      |
|-----------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Average Daily Census (ADC)</b> | The average number of patients on hospice service on any given day. The primary revenue driver for every hospice branch. Each additional patient generates roughly \$120–\$155 in daily contribution margin toward fixed costs. |
| <b>Break-Even ADC</b>             | The minimum census at which revenue exactly covers all costs — salaries, patient costs, and overhead. Below this number, the branch loses money every day. Typically 35–55 patients for a standard community-based branch.      |

|                                     |                                                                                                                                                                                                                                                               |
|-------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Target Margin ADC</b>            | The census needed to achieve the operating margin goal (typically 12–18%). This is the real sales target — not just break-even. The difference between the two is usually 15–30 additional patients.                                                          |
| <b>Contribution Margin</b>          | Revenue per patient day minus direct patient costs. Approximately \$120–\$155 per patient per day in a standard acuity branch. This is what covers salaries, overhead, and ultimately produces operating profit.                                              |
| <b>Routine Home Care (RHC)</b>      | The Medicare per diem rate paid for each day a patient is enrolled in hospice and receiving care at home. Days 1–60 are reimbursed at a higher rate than days 61 and beyond. The primary source of hospice branch revenue.                                    |
| <b>Monthly Admissions Needed</b>    | New patient enrollments required each month to maintain target ADC, calculated as $(\text{Target ADC} \times 365) \div \text{Avg. LOS} \div 12$ . This drives marketer headcount and individual production targets.                                           |
| <b>Cash Runway</b>                  | How many months starting capital can sustain a branch operating below break-even before reserves are exhausted. The key metric for new branch launch planning — your ramp timeline must reach break-even before runway runs out.                              |
| <b>Average Length of Stay (LOS)</b> | Average number of days a patient remains on hospice service before death or discharge. Longer LOS shifts revenue toward the lower Day 61+ rate but reduces required monthly admissions. LOS directly determines the admissions volume your team must produce. |

Access the Branch Profitability Simulator and full field education library at [spartancoaching.com](https://spartancoaching.com)

**Spartan Coaching | Practical coaching and consulting for hospice growth professionals**

*Every input is customizable. Every output is actionable. Start with your real numbers.*